THE NEURAL OVERRIDE PROTOCOLS
A Guide to Psychiatric Triage, Mechanics, and Reagents
NotebookLM

# THE TRIAGE GATE: ASSESSMENT

NON-PSYCHOTIC
Has Insight
Reality-Based
Protocol: Med/Surg

PSYCHOTIC
NO Insight
NOT Reality-Based
Protocol: Unique Override

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# PROTOCOL FOR THE INTACT MIND (NON-PSYCHOTIC)

Patient Status:
Insight Present +
Reality Based.

☑ THERAPEUTIC COMMUNICATION:

- "That must be overwhelming for you."
- "How are you feeling?"
- "Tell me more."
- Technique: Reflection, Clarification, Restatement.

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# MALFUNCTION I: DELUSIONS

1. PARANOID
("People are out to kill me")

2. GRANDIOSE
("I am God/The President")

3. SOMATIC
("I have X-ray vision / Worms in my arm")

Definition: A false, FIXED belief.
NO sensory component.

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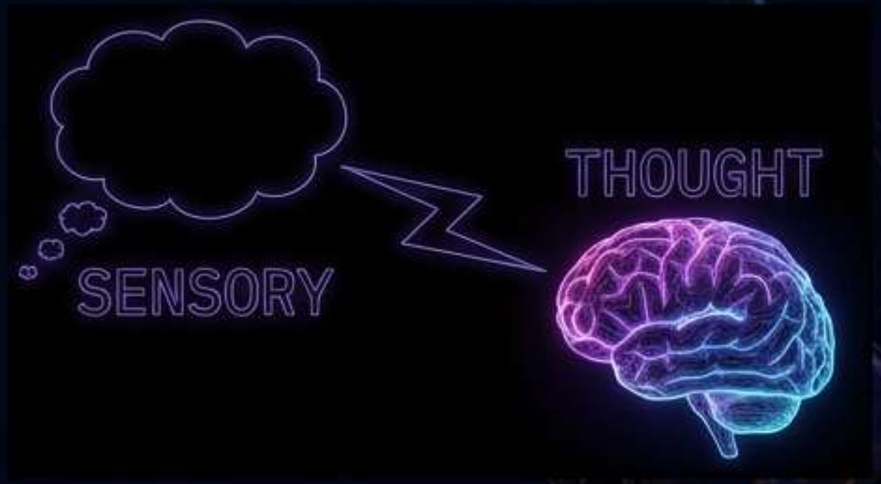
# MALFUNCTION II: HALLUCINATIONS

Definition: Sensory experience WITHOUT a referent. Nothing is there.

1. AUDITORY (Most Common - Voices)
2. VISUAL (Seeing bugs)
3. TACTILE (Feeling bugs)
4. GUSTATORY (Taste)
5. OLFACTORY (Smell)

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# MALFUNCTION III: ILLUSIONS

Definition:
Misinterpretation of reality WITH a referent.

There is something there.
The sensory input is real.
The interpretation is wrong.

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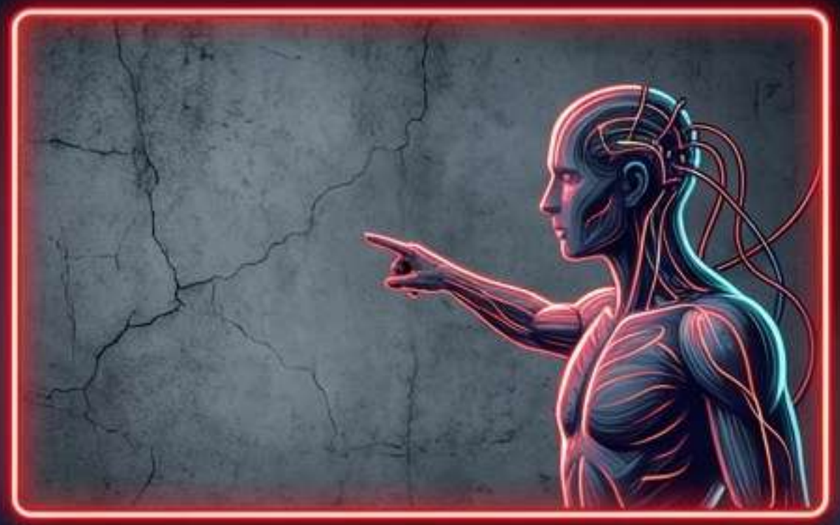
# THE CRITICAL TEST: ILLUSION VS. HALLUCINATION

Referent exists.

**ILLUSION.**



No Referent.

**HALLUCINATION.**

Question:
Is there something actually there?

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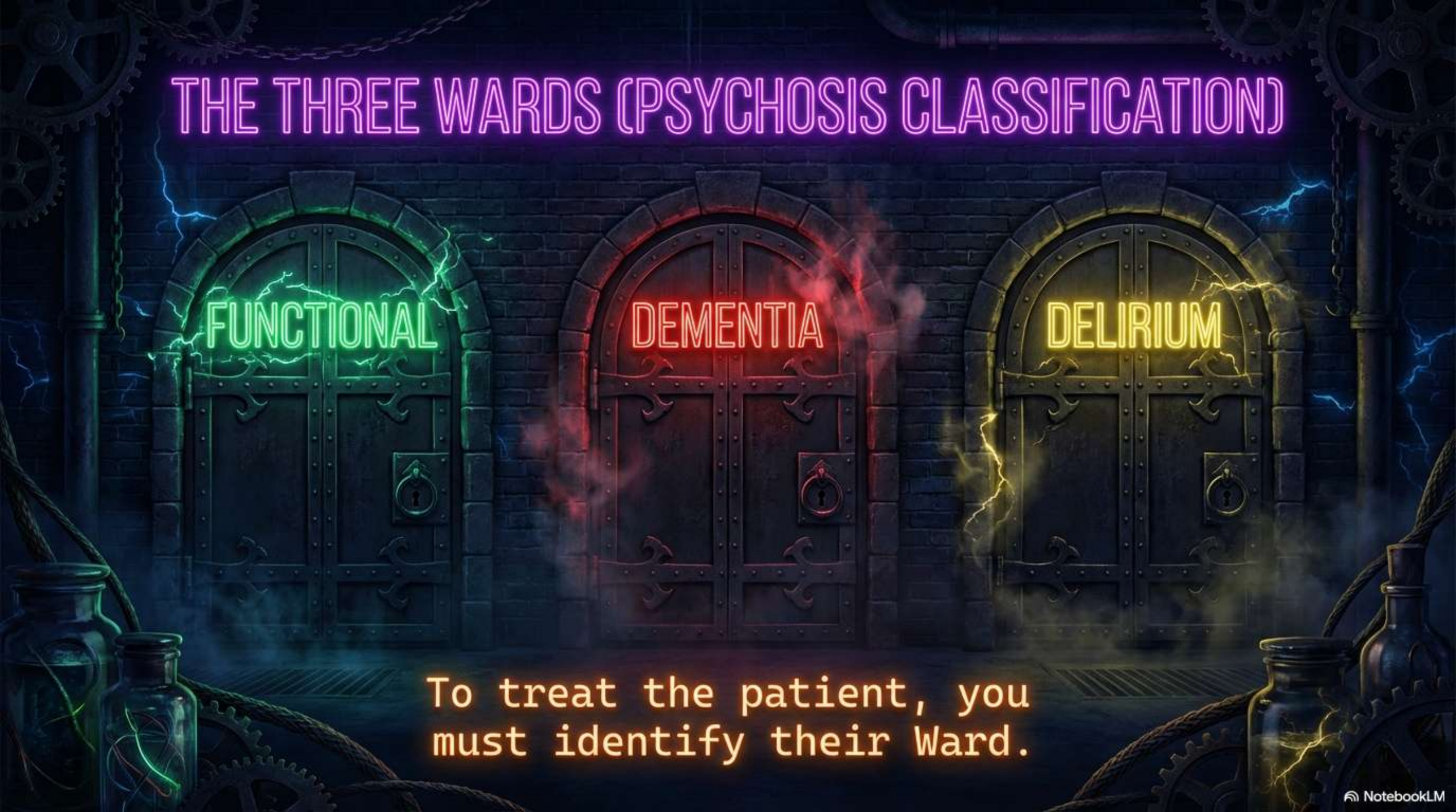
# THE THREE WARDS (PSYCHOSIS CLASSIFICATION)




To treat the patient, you must identify their Ward.

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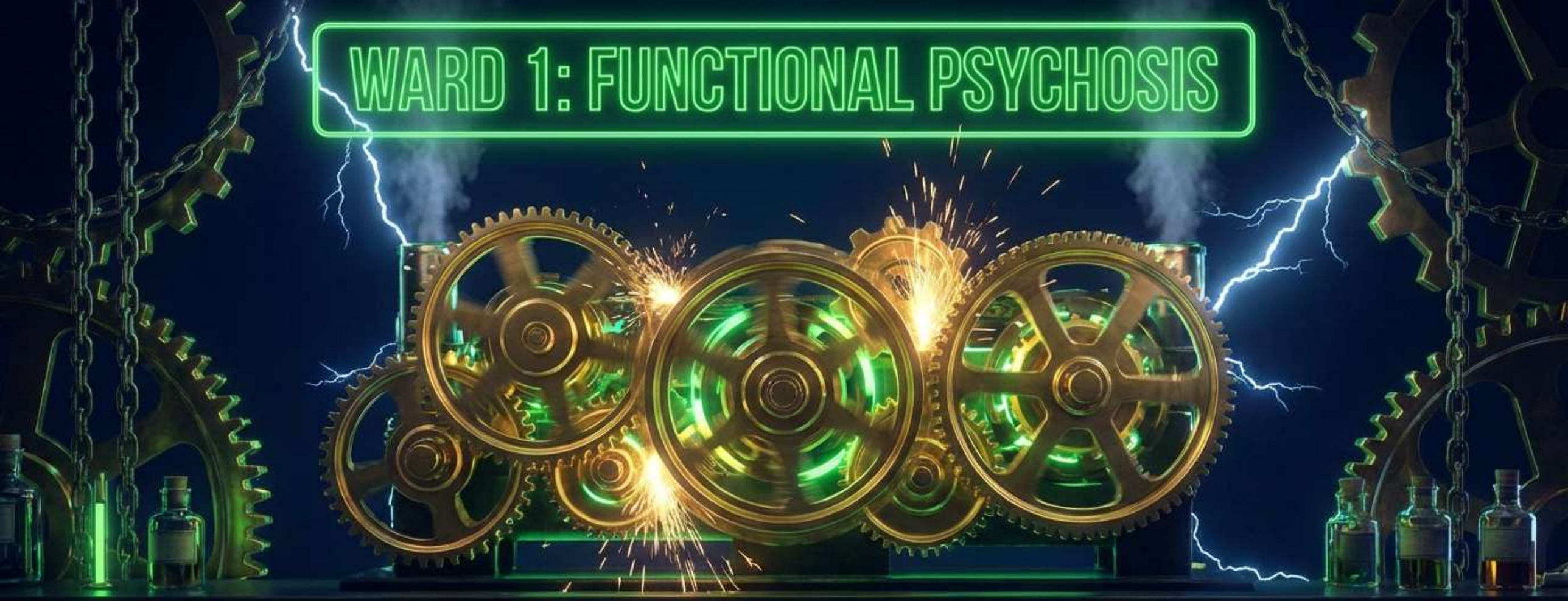
# WARD 1: FUNCTIONAL PSYCHOSIS



Diagnosis: Schizophrenia, Schizo-affective, Major Depression, Mania.

Potential: Can learn reality. Brain structure is intact. Chemical imbalance only.

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# PROTOCOL: THE FUNCTIONAL PATIENT

1. ACKNOWLEDGE FEELING
   ("You seem upset.")
2. PRESENT REALITY
   ("I know it is real to you, but I do not see a demon.")
3. SET LIMITS ("We are not going to talk about that.")
4. ENFORCE LIMITS ("Our conversation is over.")

**Goal: Teach Reality.**

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# WARD 2: PSYCHOSIS OF DEMENTIA

Diagnosis: Alzheimer's, Stroke, Organic Brain Syndrome.

Constraint: CANNOT learn reality. Brain damage is permanent.

**DO NOT PRESENT REALITY. IT WILL CAUSE FRUSTRATION.**

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# PROTOCOL: THE DEMENTIA PATIENT

1. ACKNOWLEDGE FEELING
   ("That sounds nice.")
2. REDIRECT
   ("Let's talk about church instead.")

Reality Orientation (Person, Place, Time) is allowed. Correcting delusions is not.

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# WARD 3: PSYCHOSIS OF DELIRIUM

Nature: Temporary, Sudden, Dramatic, Episodic.

Causes: UTI, Thyroid Storm, Withdrawal, Post-Op.

Protocol:

1. ACKNOWLEDGE FEELING.
2. REASSURE (Safety + Temporariness).

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# DEBRIS: LOOSE ASSOCIATIONS

FLIGHT OF IDEAS: Rapid flow of thought to thought.
WORD SALAD: Random words thrown together.
NEOLOGISMS: Made up words.
NARROWED SELF-CONCEPT: Refusal to leave room/change clothes.

Do not force. Wait until ready.

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# THE APOTHECARY: PHENOTHIAZINES

The ‘Zines’. Typical Antipsychotics.

Role: Major Tranquilizers. The Big Guns.
Small Doses = Antiemetics.
Action: Reduces symptoms. Does not cure.

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# SIDE EFFECTS: PHENOTHIAZINES (ABCDEFG)

- A - Anticholinergic (Dry Mouth)
- B - Blurred Vision
- C - Constipation
- D - Drowsiness
- E - EPS (Extrapyramidal Symptoms / Parkinsonian)
- F - Fotosensitivity
- aG - aGranulocytosis (Low WBC / Immunosuppressed)

Action: Inform patient. Treat side effects.
DO NOT HOLD DRUG (Unless Toxic).

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# THE APOTHECARY: BENZODIAZEPINES

The ‘Zeps’. Minor Tranquilizers.

Indication: Pre-op, Muscle Relaxant, Alcohol Withdrawal, Seizures.

⚠️ WARNING: Short term use only (< 2-4 Weeks). ⚠️

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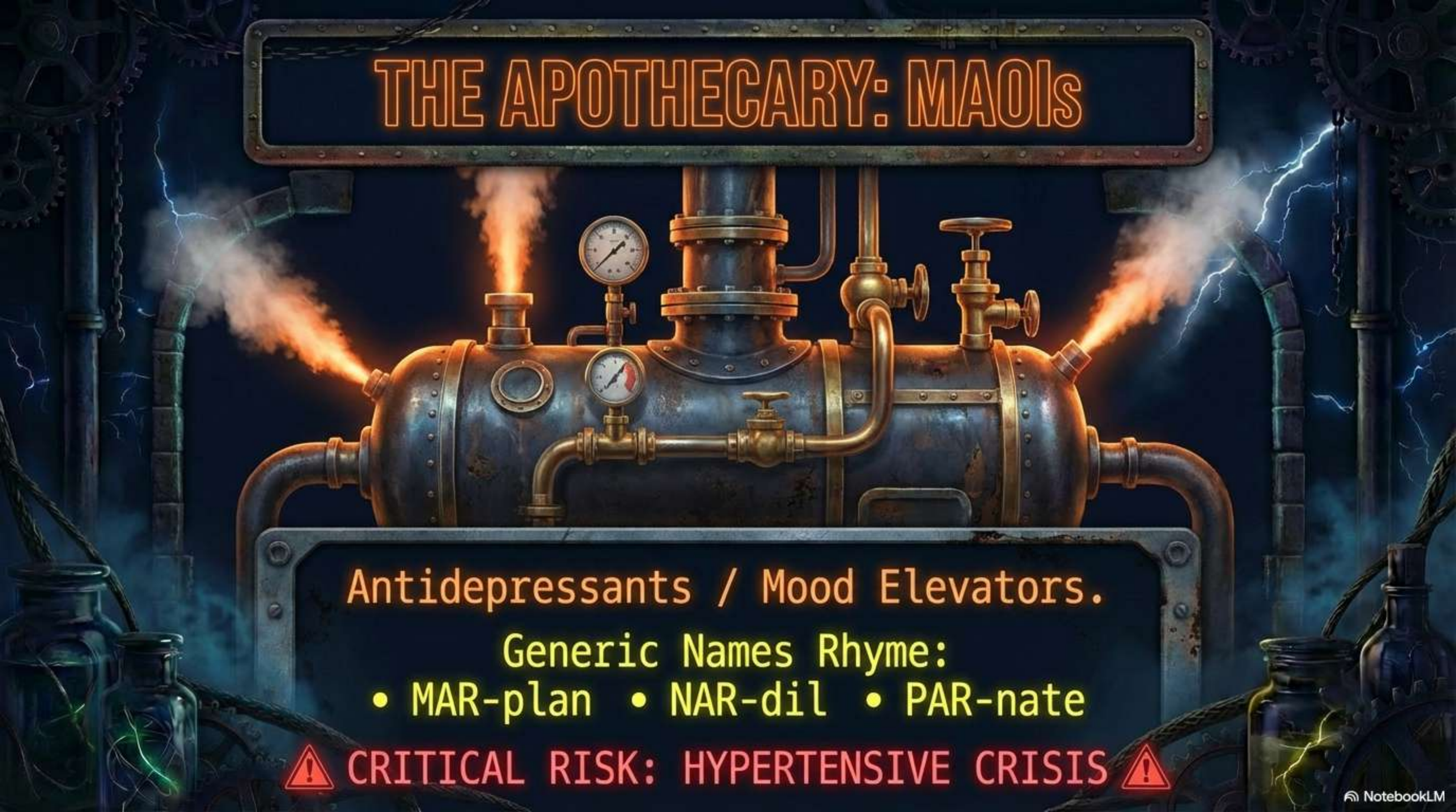
# THE APOTHECARY: MAOIs



Antidepressants / Mood Elevators.

Generic Names Rhyme:

• MAR-plan • NAR-dil • PAR-nate

⚠️ CRITICAL RISK: HYPERTENSIVE CRISIS ⚠️

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# MAOI PROTOCOL: THE TYRAMINE BAN

AVOID THE ‘SALAD BAR’:

- B – Bananas
- A – Avocados
- R – Raisins (Dried Fruit)

NO Organ Meats. NO Preserved Meats.
NO Aged Cheese. NO Yogurt/Alcohol.

SAFE: Mozzarella, Cottage Cheese.

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# THE APOTHECARY: LITHIUM

Indication: Bipolar Mania.

Side Effects (The 3 Ps):

1. Peeing (Polyuria)
2. Pooping (Diarrhea)
3. Paresthesia (Numbness/Tingling)

Intervention: Give Fluids.

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# LITHIUM TOXICITY & SODIUM

TOXIC SIGNS: Tremors, Metallic Taste, Severe Diarrhea.

The Mechanism:
LOW Sodium = TOXIC Lithium.
HIGH Sodium = INEFFECTIVE Lithium.

Action: Keep Sodium Normal.
Use electrolytes (Gatorade) for sweaters.

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